SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Ipilimumab alone — advanced melanoma

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Advanced melanoma · NCCP 00105 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
To help immune cells control advanced melanoma using ipilimumab alone; the planned infusion course is limited, but immune effects and follow-up continue beyond it.

Schedule
Ipilimumab by IV infusion once every 21 days, up to four doses. Complete within 16 weeks of the first dose, whichever limit comes first.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Ipilimumab
CTLA-4 CHECKPOINT IMMUNOTHERAPY

- ☐ Bowel inflammation and severe diarrhoea
- ☐ Liver and skin inflammation
- ☐ Thyroid, pituitary and adrenal problems
- ☐ Rare nerve, eye, muscle or heart inflammation
- ☐ Infusion reactions

Discuss the exact diagnosis and treatment intent, expected benefit and uncertainty, alternatives, regimen-specific risks, tests, fertility, practical burdens and your preferences. This guide supports the consent conversation and does not replace it. This guide describes ipilimumab monotherapy at 3 mg/kg. It is not the nivolumab combination and does not automatically lead to nivolumab maintenance. A four-dose treatment course does not mean immune risks end after dose four.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00105 v7.

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